

Package Insert

1. Product name

Aremed 1mg film-coated tablet.

2. Name and Strength of Active Ingredient

Each film-coated tablet contains 1 mg anastrozole.

Excipient(s) with known effect

This product contains 65 mg lactose.

3. Product description

Film-coated tablet.

White, round, film-coated tablets with a diameter of 6,6 mm approximately

4. Pharmacological Properties

Pharmacodynamics

Pharmacotherapeutic group: Endocrine therapy; Hormone antagonists and related agents, ATC code: L02BG03

Mechanism of action and pharmacodynamics effects

Aremed is a potent and highly selective non-steroidal aromatase inhibitor. In postmenopausal women, estradiol is produced primarily from the conversion of androstenedione to esterone through the aromatase enzyme complex in peripheral tissues. Esterone is subsequently converted to estradiol. Reducing circulating estradiol levels has been shown to produce a beneficial effect in women with breast cancer. In postmenopausal women, anastrozole at a daily dose of 1 mg produced estradiol suppression of greater than 80% using a highly sensitive assay.

Anastrozole does not possess any progestogenic, androgenic or oestrogenic activity.

Daily doses of anastrozole up to 10 mg do not have any effect on cortisol or aldosterone secretion, measured before or after standard ACTH challenge testing. Corticoid supplements are therefore not needed.

Primary adjuvant treatment of early breast cancer

In a large phase III study conducted in 9366 postmenopausal women with operable breast cancer treated for 5 years, anastrozole was shown to be statistically superior to tamoxifen in disease free survival. A greater magnitude of benefit was observed for

disease free survival in favour of anastrozole versus tamoxifen for the prospectively defined hormone receptor positive population.

Anastrozole was statistically superior to tamoxifen in time to recurrence. The difference was of greater magnitude than in disease free survival for both the Intention-To-Treat (ITT) population and hormone receptor positive population.

Anastrozole was statistically superior to tamoxifen in terms of time to distant recurrence. The incidence of contralateral breast cancer was statistically reduced for anastrozole compared to tamoxifen. Following 5 years of therapy, anastrozole is at least as effective as tamoxifen in terms of overall survival. However, due to low death rates, additional follow-up is required to determine more precisely the long-term survival for anastrozole relative to tamoxifen. With 68 months median follow-up, patients in the ATAC study have not been followed up for sufficient time after 5 years of treatment, to enable a comparison of long-term post treatment effects of anastrozole relative to tamoxifen.

ATAC endpoint summary: 5-year treatment completion analysis

Efficacy endpoints	Number of events (frequency)			
	Intention-to-treat population		Hormone receptor-positive tumor status	
	Anastrozole (N=3,125)	Tamoxifen (N=3,116)	Anastrozole (N=2,618)	Tamoxifen (N=2,598)
Disease-free survival^a	575 (18.4)	651 (20.9)	424 (16.2)	497 (19.1)
Hazard ratio	0.87		0.83	
2-sided 95% CI	0.78 to 0.97		0.73 to 0.94	
p-value	0.0127		0.0049	
Distant disease-free survival^b	500 (16.0)	530 (17.0)	370 (14.1)	394 (15.2)
Hazard ratio	0.94		0.93	
2-sided 95% CI	0.83 to 1.06		0.80 to 1.07	
p-value	0.2850		0.2838	
Time to recurrence^c	402 (12.9)	498 (16.0)	282 (10.8)	370 (14.2)
Hazard ratio	0.79		0.74	
2-sided 95% CI	0.70 to 0.90		0.64 to 0.87	
p-value	0.0005		0.0002	
Time to distant recurrence^d	324 (10.4)	375 (12.0)	226 (8.6)	265 (10.2)
Hazard ratio	0.86		0.84	
2-sided 95% CI	0.74 to 0.99		0.70 to 1.00	
p-value	0.0427		0.0559	
Contralateral breast primary	35 (1.1)	59 (1.9)	26 (1.0)	54 (2.1)
Odds ratio	0.59		0.47	
2-sided 95% CI	0.39 to 0.89		0.30 to 0.76	
p-value	0.0131		0.0018	
Overall survival^e	411 (13.2)	420 (13.5)	296 (11.3)	301 (11.6)
Hazard ratio	0.97		0.97	
2-sided 95% CI	0.85 to 1.12		0.83 to 1.14	

p-value	0.7142	0.7339
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^a Disease-free survival includes all recurrence events and is defined as the first occurrence of loco-regional recurrence, contralateral new breast cancer, distant recurrence or death (for any reason).

^b Distant disease-free survival is defined as the first occurrence of distant recurrence or death (for any reason).

^c Time to recurrence is defined as the first occurrence of loco-regional recurrence, contralateral new breast cancer, distant recurrence or death due to breast cancer.

^d Time to distant recurrence is defined as the first occurrence of distant recurrence or death due to breast cancer.

^e Number (%) of patients who had died.

As with all treatment decisions, women with breast cancer and their physician should assess the relative benefits and risks of the treatment.

When anastrozole and tamoxifen were co-administered, the efficacy and safety were similar to tamoxifen when given alone, irrespective of hormone receptor status. The exact mechanism of this is not yet clear. It is not believed to be due to a reduction in the degree of estradiol suppression produced by anastrozole.

Adjuvant treatment of early breast cancer for patients being treated with adjuvant tamoxifen

In a phase III trial (ABCSG 8) conducted in 2579 postmenopausal women with hormone receptor-positive early breast cancer who had received surgery with or without radiotherapy and no chemotherapy, switching to anastrozole after 2 years adjuvant treatment with tamoxifen was statistically superior in disease-free survival when compared to remaining on tamoxifen, after a median follow-up of 24 months.

Time to any recurrence, time to local or distant recurrence and time to distant recurrence confirmed a statistical advantage for anastrozole, consistent with the results of disease-free survival. The incidence of contralateral breast cancer was very low in the two treatment arms with a numerical advantage for anastrozole. Overall survival was similar for the two treatment groups.

ABCSG 8 trial endpoint and results summary

Efficacy endpoints	Number of events (frequency)	
	Anastrozole (N=1,297)	Tamoxifen (N=1,282)
Disease-free survival	65 (5.0)	93 (7.3)
Hazard ratio	0.67	
2-sided 95% CI	0.49 to 0.92	
p-value	0.014	
Time to any recurrence	36 (2.8)	66 (5.1)
Hazard ratio	0.53	
2-sided 95% CI	0.35 to 0.79	
p-value	0.0002	
Time to distant recurrence	22 (1.7)	41 (3.2)
Hazard ratio	0.52	
2-sided 95% CI	0.31 to 0.88	
p-value	0.015	
New contralateral breast cancer	7 (0.5)	15 (1.2)

Odds ratio	0.46	
2-sided 95% CI	0.19 to 1.13	
p-value	0.090	
Overall survival	43 (3.3)	45 (3.5)
Hazard ratio	0.96	
2-sided 95% CI	0.63 to 1.46	
p-value	0.840	

Two further similar trials (GABG/ARNO 95 and ITA), in one of which patients had received surgery and chemotherapy, as well as a combined analysis of ABCSG 8 and GABG/ARNO 95, supported these results.

The anastrozole safety profile in these 3 studies was consistent with the known safety profile established in postmenopausal women with hormone receptor-positive early breast cancer.

Study of anastrozole with the bisphosphonate risedronate (SABRE)

Bone Mineral Density (BMD)

In the phase III/IV SABRE study, 234 postmenopausal women with hormone receptor positive early breast cancer scheduled for treatment with anastrozole 1 mg/day were stratified to low, moderate and high risk groups according to their existing risk of fragility fracture. The primary efficacy parameter was the analysis of lumbar spine bone mass density using DEXA scanning. All patients received treatment with vitamin D and calcium. Patients in the low risk group received anastrozole alone (N=42), those in the moderate group were randomised to anastrozole plus risedronate 35 mg once a week (N=77) or anastrozole plus placebo (N=77) and those in the high risk group received anastrozole plus risedronate 35 mg once a week (N=38). The primary endpoint was change from baseline in lumbar spine bone mass density at 12 months.

The 12-month main analysis has shown that patients already at moderate to high risk of fragility fracture showed no decrease in their bone mass density (assessed by lumbar spine bone mineral density using DEXA scanning) when managed by using anastrozole 1 mg/day in combination with risedronate 35 mg once a week. In addition, a decrease in BMD which was not statistically significant was seen in the low risk group treated with anastrozole 1 mg/day alone. These findings were mirrored in the secondary efficacy variable of change from baseline in total hip BMD at 12 months.

This study provides evidence that the use of bisphosphonates should be considered in the management of possible bone mineral loss in postmenopausal women with early breast cancer scheduled to be treated with anastrozole.

Lipids

In the SABRE study, there was a neutral effect on plasma lipids in those patients treated with anastrozole plus risedronate.

Pharmacokinetics

Absorption of anastrozole is rapid and maximum plasma concentrations typically occur within two hours of dosing (under fasted conditions). Anastrozole is eliminated slowly with a plasma elimination half-life of 40 to 50 hours. Food slightly decreases the rate but not the extent of absorption. The small change in the rate of absorption is not expected to result in a clinically significant effect on steady-state plasma concentrations during once daily dosing of anastrozole tablets. Approximately 90 to 95% of plasma anastrozole steady-state concentrations are attained after 7 daily doses. There is no evidence of time or dosedependency of anastrozole pharmacokinetic parameters.

Anastrozole pharmacokinetics are independent of age in postmenopausal women.

Anastrozole is only 40% bound to plasma proteins.

Anastrozole is extensively metabolised by post-menopausal women with less than 10% of the dose excreted in the urine unchanged within 72 hours of dosing. Metabolism of anastrozole occurs by N-dealkylation, hydroxylation and glucuronidation. The metabolites are excreted primarily via the urine. Triazole, the major metabolite in plasma, does not inhibit aromatase.

The apparent oral clearance of anastrozole in volunteers with stable hepatic cirrhosis or renal impairment was in the range observed in healthy volunteers.

5. Clinical Particulars

Indications

Adjuvant treatment of postmenopausal women with hormone receptor positive early breast cancer.

Treatment of advanced breast cancer in post-menopausal women. Efficacy has not been demonstrated in oestrogen receptor negative patients unless they had a previous positive clinical response to tamoxifen.

Posology and method of administration

Adults including the elderly:	One 1mg tablet to be taken orally once a day.
Children:	Not recommended for use in children (see Pharmacological Properties).
Renal impairment:	No dosage change is recommended in patients with mild or moderate renal impairment.
Hepatic impairment:	No dose change is recommended in patients with mild hepatic disease.

For early disease, the recommended duration of treatment should be 5 years.

Method of administration

Oral administration.

Contraindications

Aremed is contra-indicated in:

- pregnant or lactating women.
- patients with severe renal impairment (creatinine clearance less than 20 ml/min).
- patients with moderate or severe hepatic disease.
- patients with known hypersensitivity to anastrozole or to any of the excipients as referenced in the list of excipients.

Oestrogen-containing therapies should not be co-administered with Aremed as they would negate its pharmacological action.

Concurrent tamoxifen therapy (see Interaction With Other Medicinal Products and Other Forms of Interaction).

Warnings and Precautions

Anastrozole is not recommended for use in children or in premenopausal women as safety and efficacy have not been established in this group of patients (see Pharmacological Properties).

The menopause should be defined biochemically in any patient where there is doubt about hormonal status.

There are no data to support the safe use of anastrozole in patients with moderate or severe hepatic impairment, or patients with severe impairment of renal function (creatinine clearance less than 20 ml/min).

Women with osteoporosis or at risk of osteoporosis, should have their bone mineral density formally assessed by bone densitometry e.g., DEXA scanning at the commencement of treatment and at regular intervals thereafter. Treatment or prophylaxis for osteoporosis should be initiated as appropriate and carefully monitored.

As anastrozole lowers circulating oestrogen levels, it may cause a reduction in bone mineral density with a possible consequent increased risk of fracture. The use of bisphosphonates may stop further bone mineral loss caused by anastrozole in postmenopausal women and could be considered.

This product contains lactose. Patients with rare hereditary problems of galactose intolerance, the Lapp lactase deficiency or glucose-galactose malabsorption should not take this medicine.

Interactions with other medicaments

Antipyrine and cimetidine clinical interaction studies indicate that the co-administration of anastrozole with other drugs is unlikely to result in clinically significant drug interactions mediated by cytochrome P450.

A review of the clinical trial safety database did not reveal evidence of clinically significant interaction in patients treated with anastrozole who also received other commonly prescribed drugs. There were no clinically significant interactions with bisphosphonates (see Pharmacodynamics).

Oestrogen-containing therapies should not be co-administered with anastrozole as they would negate its pharmacological action (see Contraindications).

Tamoxifen should not be co-administered with anastrozole as this may diminish its pharmacological action (see Contraindications).

Pregnancy and Lactation

Anastrozole is contraindicated in pregnant or lactating women.

Undesirable effects

Unless specified, the frequency categories were calculated from the number of adverse events reported in a large phase III study conducted in 9366 postmenopausal women with operable breast cancer treated for five years and unless specified, no account was taken for the frequency within the comparative treatment group or whether the investigator considered it to be related to study medication.

Frequency	System Organ Class	Adverse reaction
Very common ($\geq 10\%$)	<i>Vascular</i>	Hot flushes, mainly mild or moderate in nature
	<i>General</i>	Asthenia, mainly mild or moderate in nature
	<i>Musculoskeletal and connective tissue disorders</i>	Arthralgia/Joint stiffness Arthritis Osteoporosis
	<i>Nervous system</i>	Headache, mainly mild or moderate in nature
	<i>Gastrointestinal</i>	Nausea, mainly mild or moderate in nature
	<i>Skin and subcutaneous tissue</i>	Rash, mainly mild or moderate in nature
	<i>Psychiatric disorders</i>	Depression
Common ($\geq 1\%$ and $< 10\%$)	<i>Skin and subcutaneous tissue</i>	Hair thinning (Alopecia), mainly mild or moderate in nature Allergic reactions
	<i>Gastrointestinal</i>	Diarrhoea, mainly mild or moderate in nature Vomiting, mainly mild or moderate in nature
	<i>Nervous system</i>	Somnolence, mainly mild or moderate in nature Carpal Tunnel Syndrome*

		Sensory disturbances (including paraesthesia, taste loss and taste perversion)
	<i>Hepatobiliary disorders</i>	Increases in alkaline phosphatase, alanine aminotransferase and aspartate aminotransferase
	<i>Reproductive system and breast</i>	Vaginal dryness, mainly mild or moderate in nature Vaginal bleeding, mainly mild or moderate in nature**
	<i>Metabolism and nutrition</i>	Anorexia, mainly mild in nature Hypercholesterolaemia, mainly mild or moderate in nature
	<i>Musculoskeletal and connective tissue disorders</i>	Bone pain Myalgia
Uncommon ($\geq 0.1\%$ and $< 1\%$)	<i>Metabolism and nutrition</i>	Hypercalcaemia (with or without an increase in parathyroid hormone)
	<i>Hepatobiliary disorders</i>	Increases in gamma-GT and bilirubin Hepatitis
	<i>Skin and subcutaneous tissue</i>	Urticaria
	<i>Musculoskeletal and connective tissue disorders</i>	Trigger finger
Rare ($\geq 0.01\%$ and $< 0.1\%$)	Skin and subcutaneous tissue	Erythema multiforme Anaphylactoid reaction Cutaneous vasculitis (including some reports of Henoch-Schönlein purpura)
Very Rare ($< 0.01\%$)	Skin and subcutaneous tissue	Stevens-Johnson syndrome Angioedema

* Events of Carpal Tunnel Syndrome have been reported in patients receiving anastrozole treatment in clinical trials in greater numbers than those receiving treatment with tamoxifen. However, the majority of these events occurred in patients with identifiable risk factors for the development of the condition.

** Vaginal bleeding has been reported commonly, mainly in patients with advanced breast cancer during the first few weeks after changing from existing hormonal therapy to treatment with anastrozole. If bleeding persists, further evaluation should be considered.

In a large phase III study conducted in 9366 postmenopausal women with operable breast cancer treated for 5 years, ischaemic cardiovascular events were reported more frequently in patients treated with anastrozole compared to those treated with tamoxifen, although the difference was not statistically significant. The observed difference was mainly due to more reports of angina pectoris and was associated with a sub-group of patients with pre-existing ischaemic heart disease.

As anastrozole lowers circulating oestrogen levels, it may cause a reduction in bone mineral density placing some patients at a higher risk of fracture (see Warnings and Precautions). The table below presents the frequency of pre-specified adverse events in the ATAC study, irrespective of causality, reported in patients receiving trial therapy and up to 14 days after cessation of trial therapy.

ATAC study pre-specified adverse events

Adverse effects	Anastrozole (N=3,092)	Tamoxifen (N=3,094)
Hot flushes	1,104 (35.7%)	1,264 (40.9%)
Joint pain/stiffness	1,100 (35.6%)	911 (29.4%)
Mood disturbances	597 (19.3%)	554 (17.9%)
Fatigue/asthenia	575 (18.6%)	544 (17.6%)
Nausea and vomiting	393 (12.7%)	384 (12.4%)
Fractures	315 (10.2%)	209 (6.8%)
Fractures of the spine, hip, or wrist/Colles	133 (4.3%)	91 (2.9%)
Wrist/Colles fractures	67 (2.2%)	50 (1.6%)
Spine fractures	43 (1.4%)	22 (0.7%)
Hip fractures	28 (0.9%)	26 (0.8%)
Cataracts	182 (5.9%)	213 (6.9%)
Vaginal bleeding	167 (5.4%)	317 (10.2%)
Ischaemic cardiovascular disease	127 (4.1%)	104 (3.4%)
Angina pectoris	71 (2.3%)	51 (1.6%)
Myocardial infarct	37 (1.2%)	34 (1.1%)
Coronary artery disorder	25 (0.8%)	23 (0.7%)
Myocardial ischaemia	22 (0.7%)	14 (0.5%)
Vaginal discharge	109 (3.5%)	408 (13.2%)
Any venous thromboembolic event	87 (2.8%)	140 (4.5%)
Deep venous thromboembolic events including PE (pulmonary embolism)	48 (1.6%)	74 (2.4%)
Ischaemic cerebrovascular events	62 (2.0%)	88 (2.8%)
Endometrial cancer	4 (0.2%)	13 (0.6%)

Fracture rates of 22 per 1000 patient-years and 15 per 1000 patient-years were observed for the anastrozole and tamoxifen groups, respectively, after a median follow-up of 68 months. The observed fracture rate for anastrozole is similar to the range reported in age-matched postmenopausal populations. It has not been determined whether the rates of fracture and osteoporosis seen in ATAC in patients on anastrozole treatment reflect a protective effect of tamoxifen, a specific effect of anastrozole, or both.

The incidence of osteoporosis was 10.5% in patients treated with anastrozole and 7.3% in patients treated with tamoxifen.

Symptoms and treatment of overdose

There is limited clinical experience of accidental overdose. In animal studies, anastrozole demonstrated low acute toxicity. Clinical trials have been conducted with various dosages of anastrozole, up to 60 mg in a single dose given to healthy male volunteers and up to 10 mg daily given to postmenopausal women with advanced breast cancer; these dosages were well tolerated. A single dose of anastrozole that results in life-threatening symptoms has not been established. There is no specific antidote to over-dosage and treatment must be symptomatic.

In the management of an overdose, consideration should be given to the possibility that multiple agents may have been taken. Vomiting may be induced if the patient is alert. Dialysis may be helpful because anastrozole is not highly protein bound. General supportive care, including frequent monitoring of vital signs and close observation of the patient, is indicated.

6. Pharmaceutical Particulars

Storage conditions

Do not store above 30 °C. Protect from light and moisture.

Shelf-life

5 years.

Pack size available

Cardboard box containing Aluminium-PVC blisters of 14 film-coated tablets.
Pack size of 28 film-coated tablets.

7. Manufacturer

Remedica Ltd.
Aharnon Street, Limassol Industrial Estate,
3056 Limassol, Cyprus

8. Product registration holder

Eucogen Sdn. Bhd.,
No. 6A, Jalan Sungai Burung U 32/U, Bukit Rimau Seksyen 32, 40460 Shah Alam
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9. Date of revision of the text

04/2024

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